ROSUVASTATIN

Asian patients: start 5 mg

Myopathy risk

CHEMICAL NAME (3R,5S,6E)-7-[4-(4-fluorophenyl)-2-(N-methylmethanesulfonamido)-6-(propan-2-yl)pyrimidin-5-yl]-3,5-dihydroxyhept-6-enoic acid (calcium)

CLASS HMG-CoA reductase inhibitor (statin), high intensity

TRADE NAMES Crestor · Rosuvas · Rosulip

THERAPEUTIC USES / INDICATIONS

- Hypercholesterolaemia and mixed dyslipidaemia.
- Prevention of cardiovascular events.
- Familial hypercholesterolaemia (incl. children ≥6 yr).
- Slowing atherosclerosis progression.

ADMINISTRATION

- **PO** once daily at any time, with or without food.

DOSAGES

- **Adult:** 5–20 mg once daily; max 40 mg (specialist, high risk only).
- **Asian patients, elderly, mild renal impairment:** start 5 mg; avoid 40 mg.
- **CrCl <30:** max 10 mg (contraindicated in some labels).
- With ciclosporin: max 5 mg.

PHARMACOKINETICS

- **Peak:** 3–5 h. Bioavailability ~20%.
- Hydrophilic; minimal CYP metabolism (CYP2C9 minor).
- Excreted mainly in faeces. **t½:** 19 h.

PHARMACODYNAMICS

- Most potent statin per mg: inhibits **HMG-CoA reductase** → ↓ LDL-C up to 55–63%.
- ↑ HDL; ↓ triglycerides.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Myalgia, headache.
- Abdominal pain, constipation, nausea.
- Asthenia.

ADVERSE EFFECTS (SERIOUS)

- Myopathy, rhabdomyolysis (dose-related, higher in Asians).
- Proteinuria (high dose), haematuria.
- Hepatitis; new-onset diabetes.

CONTRAINDICATIONS & CAUTIONS

- Active liver disease.
- Pregnancy and breastfeeding.
- Severe renal impairment (CrCl <30) for 40 mg.
- Use with ciclosporin (high doses); myopathy.

KEY INTERACTIONS

- **Ciclosporin, gemfibrozil:** ↑ levels → myopathy.
- **Warfarin:** ↑ INR.
- **Antacids (Al/Mg):** ↓ absorption — separate by 2 h.
- **Fusidic acid:** rhabdomyolysis.

NURSING MANAGEMENT

- Baseline and 3-month lipids, LFTs, renal function.
- Ask about muscle symptoms; CK if present.
- Urine dipstick for protein at high doses.
- Check ethnicity and renal function for starting dose.

PATIENT EDUCATION

- Take daily; continue lifestyle changes.
- Report muscle pain, weakness, dark urine.
- Avoid pregnancy.
- Separate from antacids.

KEY POINTS

- Most potent statin per mg.
- Mostly not CYP-metabolised.
- Start 5 mg in Asians and renal impairment.
- Myopathy; proteinuria at 40 mg.